

1.

CASE #	CASE NAME	HEARING NAME
CVRI2504507	AMARAL-HERNANDEZ VS MDI INTERIORS	MOTION TO SET ASIDE DEFAULT

Tentative Ruling:

The motion is granted. The two entries of default are vacated. The defendants shall serve their answers no later than 7-2-26.

The plaintiff's request for sanctions is denied.

The order to show cause returnable on 7-7-26 is discharged. The case management conference is vacated. A trial setting conference shall be conducted on 10-26-26 at 8:30 A.M. in this department. Before that date, counsel are ordered to meet and confer to select two trial dates that are agreeable to all parties. The dates selected must be on Fridays no more than six months after the TSC.

Analysis:

The showing of mistake, inadvertence, surprise, or excusable neglect, though not compelling, is sufficient. Moreover, the plaintiff does not claim that he would be prejudiced by the relief sought by the defendants. He certainly offers no evidence of any such prejudice. Less than a month – 24 days after the entry of one defendant's default, and only 10 days after the other – passed between the entry of the defaults and the defendants' request that the that the plaintiff stipulate to the vacation of the defaults. Under these circumstances, the policy in favor of litigating legal controversies on their merits prevails.

2.

CASE #	CASE NAME	HEARING NAME
CVRI2506653	DAI VS CORONA REGIONAL MEDICAL CENTER LLC	DEMURRER ON 1ST AMENDED COMPLAINT

Tentative Ruling:

The demurrer to the second cause of action of the first amended complaint is sustained. Further leave to amend is denied unless the plaintiff explains how the FAC can be amended to allege the necessary custodial relationship and custodial neglect.

Analysis:

To plead elder abuse by neglect, "[t]he plaintiff must allege (and ultimately prove by clear and convincing evidence) facts establishing that the defendant: (1) had

responsibility for meeting the basic needs of the elder or dependent adult, such as nutrition, hydration, hygiene or medical care [citations]; (2) knew of conditions that made the elder or dependent adult unable to provide for his or her own basic needs [citations]; and (3) denied or withheld goods or services necessary to meet the elder or dependent adult's basic needs" either by means of oppression, fraud or malice; or recklessness. (*Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, 407.) Moreover, "the facts constituting the neglect and establishing the causal link between the neglect and the injury 'must be pleaded with particularity,' in accordance with the pleading rules governing statutory claims." (*Ibid.*) Here, Plaintiff has failed to plead a cause of action for elder abuse with the requisite particularity.

As a threshold showing, neglect under the Elder Abuse Act requires that there first be a caretaking or custodial relationship between plaintiff and defendant. (*Winn v. Pioneer Medical Group, Inc.* (2016) 63 Cal.4th 148, 161; Welf. & Inst. Code §15610.57(a)(1).) "[C]laims under the Elder Abuse Act are not brought against health care providers in their capacity as providers but, rather, against custodians and caregivers that abuse elders and that may or may not, incidentally, also be health care providers." (*Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, 786.) The type of caretaking or custodial relationship that the Elder Abuse Act requires is "one where a party has accepted responsibility for attending to the basic needs of an elder or dependent adult," which an able-bodied and fully competent adult would ordinarily be capable of handling on his or her own. (*Ibid.*) As the Supreme Court explained: "[N]othing in the legislative history suggests that the Legislature intended the Act to apply whenever a doctor treats any elderly patient. Reading the act in such a manner would radically transform medical malpractice liability relative to the existing scheme." (*Winn, supra*, 63 Cal.4th at 163.) Plaintiff's FAC and Opposition focus on how Dr. Bhutwala was allegedly negligent in failing to render medical care and treatment to him. However, Plaintiff fails to establish the preliminary requirement of a "robust caretaking and custodial relationship" between Dr. Bhutwala and himself, which must exist before the Court can even consider whether Dr. Bhutwala's actions or omissions constitute neglect. Specifically, Plaintiff must allege specific facts to show, not merely conclude, that Dr. Bhutwala "assumed a significant measure of responsibility for attending to one or more of an elder's basic needs that an able-bodied and fully competent adult would ordinarily be capable of managing without assistance," which requires allegations of a robust relationship beyond mere provision of professional medical services. (*Winn, supra*, 63 Cal.4th at 158.)

Even if Plaintiff established that a custodial or caretaking relationship existed between Dr. Bhutwala and himself, the FAC fails to allege custodial neglect, and as such, is not sufficient to support a cause of action for neglect under the Elder Abuse Act. To do so, the Plaintiff must allege that Dr. Bhutwala's actions or omissions arose in his capacity as Plaintiff's custodian and provider of basic necessities, including personal hygiene, food, hydration, clothing, and safe living space. (*Holland v. Silverscreen Healthcare, Inc.* (2025) 18 Cal.5th 364, 376; *Faiaipau v. THC-Orange County, LLC* (2025) 117 Cal.App.5th 292, 304.)